Stage 5 2024-2025 - Compiled Questions / Recalls

Answer-complete QA pass. Answers were taken from explicit source answers/notes where visible; items with no visible answer are marked; duplicate/near-duplicate recalls were consolidated.

Dermatology (17 questions)

#	Fact cue / question	Answer
1	Child with oozing and crustations on cheeks?	Subacute eczema
2	lip licker dermatitis	ICD
3	Papules on the rough surface of the hand?	DNA Papo virus
4	Child with symptoms; u cat at home	tinea capitis
5	Zoster case; treat within?	48-72 hrs
6	atopic dermatitis remission	summer due to heliotherapy
7	soldier case with pustule and papule in the beard	folliculitis barbe
8	typical case of telogen effluvium, which one is wrong?	pull test is negative
9	A male worker?	Tinea incognito
10	patient using antihistamine now presented with BPH, what does it lead to?	Difficulty in urination
11	Whats wrong about scabies?	Secondary manifestations are rare
12	Case of acne vulgaris: pustule, papule, without nodules	Topical + Doxcylcine ?
13	case of vitiligo	topical steroid
14	History of rosacea	papulopustular rosacea
15	asked about a test; it was a hair disorder	KOH TEST
16	A 12 yrs old girl, short broken hair of varying length?	trichotillomania
17	Multiple patch?	Ana

Family medicine (9 questions)

#	Fact cue / question	Answer
18	commonest risk factor for obesity?	genetic
19	Correct abt DM?	Fpg and hba1c to screen
20	Guy who smokes and doesn’t want to quit?	Motivational interviewing
21	family medicine case: high lipid profile and CAD risk?	Start intense drug therapy and lifestyle therapy immediately
22	1st line for bloody diarrhea?	Ciprofloxacin
23	chronic fatigue, examination unremarkable; which investigation is necessary?	Thyroid test
24	10 month old child with RR 52 bpm, no other symptoms. Diagnosis?	Pneumonia
25	mother delivered a baby 1 month ago; on check up she wasn't so bright with the child	all above
26	case about OCP and backup methods	if it's taken within the 5 days of menstruation then there's no need for backup

Rheumatology (10 questions)

#	Fact cue / question	Answer
27	Most specific for SLE?	Anti-sm
28	Drug of choice for RA among DMARD?	methotrexate
29	Hallmark of RA	morning stiffness more than 1 hr
30	Acute gout	Colchicine
31	Gout presentation is in the?	1st metatarsophalyngeal joint
32	a drug that causes SLE-like syndrome?	Aspirin
33	Case of SLE, which is characteristic?	all of the above
34	Rheumatology EMQ item 32	SLE
35	Rheumatology EMQ item 33	Polymyalgia rheumatica
36	Rheumatology EMQ item 34	RA

Nephrology (16 questions)

#	Fact cue / question	Answer
37	Which preceding infection is associated with post-infectious glomerulonephritis?	Impetigo after one week
38	indications for dialysis, doesn’t include	Hypercalcemia
39	Complications of nephrotic syndrome?	Thromboembolism
40	Most common cause of nephrotic syndrome in pediatrics	minimal change disease
41	Most common kidney stone?	calcium oxalate
42	earliest sign of diabetic nephropathy?	Microalbuminuria
43	Fanconi syndrome	proximal tubule
44	Which one is contraindicated in bilateral renal artery stenosis?	ACE inhibitors
45	EPO therapy in CKD can cause?	hypertension
46	Which causes acute interstitial nephritis?	Ibuprofen
47	45 yo female, weight gain, pedal edema, HTN	Cushing syndrome
48	BP very high, emergency department, papilledema…	Emergency hypertensive
49	73 yo hypertensive, peripheral vascular disease, heavy smoker	renal artery stenosis
50	Tremor	uremic encephalopathy? (uncertain)
51	Bleeding with normal plt?	Platelet dysfunction
52	Bone pain, high PTH	secondary hyperparathyroidism

Hematology (13 questions)

#	Fact cue / question	Answer
53	About follicular lymphoma, which one is incorrect?	It is common in our area
54	Prolonged bleeding time 12 min, which one is not correct	Factor XI deficiency
55	question likely AML; which one was wrong	Send for t(9 :22)
56	old man with progressive pallor and back pain, kidney involved; bone marrow biopsy will show?	plasma cell infiltration
57	Patient with venesection that changed to anemia now, needs transfusion and has huge splenomegaly	myelofibrosis
58	Case with frequent blood transfusion; what was the father's blood	HbA2
59	which chemotherapeutic drug causes acute pancreatitis?	Asparagunase
60	Guy with brother had splenectomy, but abnormal morphology still exists?	Hereditary spherocytosis
61	which one doesn't occur in IDA	increase reticulositosis
62	wrong abt anticoagulants?	Dabigatran is an X inhibitor
63	30 yo presents with fever, bruises, no lymphadenopathy or hepatosplenomegaly, Hb 8, thrombocytopenia, leukopenia, normal bleeding time and PTT	Aplastic anemia
64	30 yo with symptoms of AML findings (10% blast, 80% promyeloblast)	AML M3
65	63-years old female with recurrent epistaxis, prolonged bleeding time and aPTT	VWD

Geriatrics (4 questions)

#	Fact cue / question	Answer
66	WHO diagnostic T-score cutoff for osteoporosis	≤ -2.5
67	Physiology of aging?	Increase diastolic
68	Leons theory about successful aging?	Most elderly people with high satisfaction in their life and a desire to learn new things
69	question about skin changes in dermatology?	Albinism (uncertain)

Pediatrics (48 questions)

#	Fact cue / question	Answer
70	Which is wrong for a case of bowed legs?	Inc serum phosphate
71	3 year old	3 word sentence
72	How to diagnose lobar emphysema?	Plane x ray
73	case about NEC, what's the radiological finding	Pneumatosis intestinalis
74	What u don't give in 18 months child?	Measles
75	positive baby and mother blood group, positive Coombs test	exchange transfusion
76	Wrong abt intussusception?	Most common in left colic flexure
77	criteria for diagnosis of infantile colic	Wessel's criteria
78	cyanosed legs but hands are pink (differential cyanosis)	CoA
79	Causes hematuria but no RBC?	beet ingestion
80	child with DM	Tell the family it is autoimmune genetics and environmental factors
81	Convulsion and diarrhea	Shigella
82	case of CP with scissoring of both legs and hypereflexia	Diplagic spasticity
83	a premature baby delivered by C/S, 800 grams; what's the 1st thing to concern?	Cold injury; RDS (mostly); hypoglycemia (uncertain — source lists all three)
84	Complication of pneumonia?	Respiratory failure; sepsis
85	Blue breath holding spell then seizure	EEG?; Iron profile? (uncertain)
86	sickle cell anemia presented with hemiparesis, acute management?	exchange transfusion
87	Which one false about HUS?	Always preceeded by diarrhea
88	Strawberry tongue	scarlet fever
89	baby at the age of 2 years, what he can do?	— no explicit answer visible in source
90	14 years girl with amenorrhea and short stature?	Growth hormone or bone age?; Karyotyping (uncertain — source lists both)
91	Case neonate increasing in cyanosis, cardiomegaly, plethoric lung	TGA
92	Roseola infantum	HHV 6
93	case of malnutrition, low protein low calories, height not affected	moderate acute malnutrition
94	Causative organism of acute epiglottitis?	Haemophilus influenza B
95	ECG shows ST depression indicates which electrolyte abnormality	HypoKalemia
96	causes of respiratory failure in the neonate includes all except?	Thyroglosul duct cyst
97	emphysema	plain Xray
98	3 year boy interacting with peers and knows some words; what to do?	reassure the parent and reassess after 6 months
99	most common treatment of bronchiolitis?	O2 therapy; supportive care (uncertain — source lists both)
100	Patient with rubella, common defect?	Persistent ductus arterosis
101	Repeated vomiting in a child?	Chest Xray
102	which one indicate malrotation	— no explicit answer visible in source
103	diagnosed case of cow milk protein allergy?	Hydrolyzed protein Formula
104	Child drooling	rolandic
105	Coryza	measles
106	diarrhea + tenderness + abdomen distended + hard life and eating problem	Celiac disease
107	Kid with meningeal signs, inappropriate answers to questions	Immediate abs then Ct then do lp if Ct is normal
108	most common cause of acute flaccid paralysis?	Gullian bare syndrome
109	horse shoe kidney	Turner Syndrome
110	low set ears, beaked nose and oligohydramnios	Potters sequence
111	single palmar crease	Downs syndrome
112	Diarrhea and some dehydration, 10 kg	750 ml per 4 hr
113	ORS for 7 month infant	50-100 ml for each bowel motion
114	Severe dehydration, 8 kg, first hour?	240 ml
115	child had abdominal pain, nausea, and after 24 hrs increased liver enzymes; what's the cause?	acetaminophen poisoning
116	child drank some cleansing agent, had drooling; what's the cause?	Caustic poisoning
117	case with heater exposure; what's the cause?	Carbonmonoxide poisoning

Ophthalmology (18 questions)

#	Fact cue / question	Answer
118	painless swelling on the upper eyelid	Chalozion
119	case about bilateral diffuse red eye with crustation over the lid	Anterior staphylococcus blepheritis
120	Case old age presented with blurry vision with inward eyelashes	Trichiases
121	Typical case of irits with inflammatory cells and keratic deposition	Iritis
122	Treatment of iritis	Corticosteroid? Oral antibiotic? (uncertain)
123	Regarding the treatment of amblyopia	Patching the good eye and training the bad eye
124	Patient with convergence, whats the most significant examination	Cycloplagic refraction
125	Fibrovascular growth nasal side	pterygium
126	Which of the following is not used as a treatment for chemical conjunctivitis?	Vitamin (a)
127	Indication for thyroid surgery	optic nerve compression
128	squamous cell carcinoma of conjuctive? tx	Excision plus chemo
129	How do you differentiate between diabetic CN III palsy and aneurysmal CN III palsy?	dilated pupil
130	Case of adult inclusion	Tetracycline ointment
131	Case of Herpes simplex	Acylclovir ointment
132	Case of allergic conjunctivitis	Olopatadine
133	70-year-old female with headache and nausea and proptosis more in right, with papilledema after a severe oral infection	Cavernous sinus thrombosis
134	A 4-year-old child with left eye proptosis (chemosis) and loss of eye movement after an upper respiratory tract infection	Orbital cellulitis
135	A young man was in an RTA and had normal vision, but developed diplopia after reading and watching his mobile	Orbital floor fracture

ENT (8 questions)

#	Fact cue / question	Answer
136	Watery discharge from nose	allergic rhinitis
137	All are signs of ear except	presbyopia
138	Old age unilateral mass in nose	inverted papilloma
139	Anterior ethmoid presents bulge in middle meatus called	haitus semilunaris
140	Flu and common cold?	Unlike the common cold, the onset of symptoms for flu or influenza is very sudden and fast.
141	most common site of allergic polyp	ethmoid sinus
142	CSF rhinorrhea is characterized by all of the following except	Sediment formation after standing a tube
143	Which of the following nerves doesnt supply the external ear?	Corda tympani

Orthopedics (19 questions)

#	Fact cue / question	Answer
144	The main artery that supplies the head and neck of the femur	Medial circumflex femoral artery
145	A young patient with a painful shoulder after overpricing, difficulty in elevating it, responding to conservative treatment	Acute calcific cuff tendinitis
146	A child with lateral Condylar fracture displaced 5 mm	ORIF with K wires
147	Common hip dislocation	posterior
148	Case with left forearm fracture, best assessment for radial nerve injury	active extension of the wrist joint
149	Shoulder dislocation	Axillary nerve damage
150	case of clavicle fracture minimally displaced	Analgesia and sling
151	a child with sudden inability to walk and left hip joint pain with increased WBC and ESR, next step	Joint aspiration; immediate antibiotics
152	Which of the following is the treatment for cubitus varus?	Wedge osteotomy with the base located laterally
153	20-year-old with old injury/fracture now has cubitus valgus and pain and tingling over the medial one and a half fingers; how do you treat it?	Anterior transfer of ulnar nerve
154	pain over the radial styloid with positive Finkelstein sign	De Quervain's Tenosynovitis
155	DDH	In clinic Ortolani and Barlow test should be done only one time not more because of hip joint injury
156	A 4 year old child presented with sudden inability to walk, hip pain, limited all range of motions, constitutional symptoms, X ray has no finding	septic arthritis
157	EMQ item: middle clavicle minimal displaced	armsling and analgesic
158	EMQ item: anterior shoulder dislocation, displaced	ORIF
159	EMQ item: undisplaced proximal shaft fracture	U shape sling
160	EMQ item: excentric radiolucent	chondromexoid
161	EMQ item: epiphisis	Chondroblastoma
162	EMQ item: strow colured in aspiration	simple bone cyst

Urology (18 questions)

#	Fact cue / question	Answer
163	question about stone: which one is wrong about stone?	Highest rate among white females
164	Not initial treatment in patient with ureteric colic	anticoagulant so as to stop clot formation
165	Wrong regarding VUR?	grade 3 clubbing of fornices
166	Wrong?	3/4th treated VUR are boys
167	Acute retention except	acute cystitis
168	Cause chronic retention except	urethral stone
169	child after football trauma with left loin pain, swelling, bruises, and first voided urine is red; best investigation	CT with contrast
170	Complication of chronic except	bladder rupture
171	Not true about chronic urinary retention	Complete inability to urinate
172	All of them differential diagnose of left uretric colic except	Acute appedcitis
173	25-year-old man with sexual contact 2 weeks ago, fever, rigor, suprapubic pain, and unable to void	Acute Prostatitis
174	Which one is not indication for admission in patient with uretric stone?	Not responding to analgesia after 1/2 hour
175	pregnant female with left colic pain	admission and analgesia with follow up
176	30-year-old male with single kidney presents with anuria, hydronephrosis, high serum creatinine and urea; what will do?	admission, investigation and endoscopy
177	female with positive family history of stone passed uric acid stone	allopurinol with potassium acetate
178	16 year old male with acute high grade fever (or no fever?), left testicular tenderness, and typical feature of torsion	left testicular torsion
179	Patient with gradual increasing size, fever, swelling, mild tenderness for about 3 days after URTI	epididimo orchitis (uncertain)
180	Patient had trauma to the left scrotum 3 months ago, no changes in the skin and not painful	hematoma or left seconday hydrocele? (uncertain)

Neurology (19 questions)

#	Fact cue / question	Answer
181	Chronic Inflammatory Demyelinating Polyradiculoneuropathy (CIDP) is best diagnosed by	EMG & NCS
182	Criteria of ILAE includes all of the following except	history of febrile seizure
183	Definition of seizure	hyperactivity in brain (exact words not remembered) (uncertain)
184	preventing future attack of cluster headache	verapamil
185	A patient cannot inform and express speech	motor dysphasia
186	65 years old patient with diabetes, LDL 190, ataxia, frequent falls, headache; suspicion of TIA, treatment	clopidogril and rosuvastatin
187	A patient with migraine using triptan for 6 days in a week and ibuprofen daily, still increased attacks; most appropriate next step in management	stop all acute medications
188	which one present in GBS	ascending paralysis
189	Multiple sclerosis	internuclear ophthalmoplegia
190	the only FDA approved therapy for Dystrophinopathies	eteplirsen
191	case with Bitemporal Hemianopia: lesion in	Optic chiasm
192	50 years vegan patient with neurological symptoms and dorsal column lesion on MRI; treatment	IM vitamin B12
193	which one is wrong about lacunar stroke	Global dysphasia
194	EMQ item: subarichnoid horrhage	nimodipine
195	EMQ item: severe htn become weaknees	labetalol
196	lady with transient left eye blindness, CT normal, no internal carotid problems; management	aspirin
197	EMQ item 80	Status epilipticus
198	EMQ item 81	Brain abscess
199	EMQ item 82	Idiopathic intracranial hypertention

Neurosurgery (14 questions)

#	Fact cue / question	Answer
200	Uncal herniation	ipsilateral 3rd CN
201	schiatica commonly due to	disc prolapse
202	effect of local ansthesia	numbness at specific area
203	Single suture cause of prominent forehead and occiput scaphocephaly	sagittal suture; Lambdoid? (uncertain — source lists both)
204	Corticospinal tract: which one is wrong?	Foot drop
205	case with high ICP features causing some nerve palsys	External ventricuar drain
206	Wrong about anterior cord syndrome	In elderly because of degenerative disease compressing spinal cord
207	Patient with increased ICP, what is the earliest radiological finding on CT scan?	Obliteration of the sulci
208	general ansthesia cause	sleep and loss of pain uring surger
209	case of truncal ataxia with cerebellar tumour pressing the fourth ventricle; cause of presentation	Non-communicating hydrocephalus
210	elderly male with a history of fall 2 weeks ago, confusion and headache	chronic subdural hematoma
211	Which of the following is most correct/relevant anatomically?	C5/C6 is most flexible one and is most common site for cervical herniation (uncertain — source says Answer:?)
212	Which of the following is false about corpus callosum?	Separation of the lateral ventricle (?) (uncertain)
213	question about what’s wrong about cranocyntosis	No definite answer specified; options recalled: MRI detects bone sclerosis; technetium bone scan (uncertain)

Psychiatry (21 questions)

#	Fact cue / question	Answer
214	autism prognosis: ratio that can participate in school and work later	10-20%
215	case with anxiety of firearm in home	Specific phobia; OCD? (uncertain — source lists both)
216	Whats the etiology of ADHD?	dopaminergic transmitter system, including the dopamine D4 and the dopamine transporter
217	patient with typical features of MDD and delusion of guilt; diagnosis	MDD with psychotic feature mood congurent
218	18 y/o visual hallucination, fluctuating conscious level with signs of parkinsonism	lewy body
219	24 year female 160 cm and 95 kg, which antipsychotic used	Aripiprazole
220	Treatment of nightmares and hypervigiliance	Prazosin
221	case with eccentric imagination with odd thinking and behavior features of schizotypal	schizotypal
222	case young female with typical feature avoidant	avoidant personality
223	case of opiate withdrawal features; what to give to treat the symptoms and prevent recurrence	Methadone
224	case of alcohol and now presented with anxiety and insomnia, last drink was 12 hours ago; what is used to treat those withdrawal symptoms	Chlodiazepoxide? (uncertain — source says “or?”)
225	Emergency case of psychiatry and descalation was not successful; what to do next	Give Resperidone orally and wait for 30 mins
226	male wants to wear female clothes to get aroused; diagnosis	Tansvestic fetishizm
227	Which of the following is a side effect of lithium	Fine tremor
228	university student with daytime sleepiness and features of cataplexy; diagnosis	Narcolepsy
229	EMQ item 112	cheonic Fatigue; somatic symptom
230	EMQ item: young male that good in hospital	Malingering, Factitious
231	EMQ item: suddenly cant walk	Factitious disorder
232	EMQ dr mahmoos item 115	Shizoaffective
233	EMQ dr mahmoos item 116	Mania
234	EMQ dr mahmoos item 117	2nd generation anti phycitic

Sara Taras Drive 2023-2024 - Deduplicated Questions / Recalls

Answer-complete QA pass. Answers were taken from visible highlighted/red/blue/circled/arrowed marks and end-of-recall notes; duplicate/near-duplicate recalls were consolidated.

Ophthalmology (52 questions)

#	Fact cue / question	Answer
1	Adult inclusion conjunctivitis treatment	Tetracycline ointment
2	Adult inclusion conjunctivitis: not true statement	Transmitted by fly
3	Advanced hypertensive retinopathy finding	Optic disc/optic nerve swelling
4	Anatomical landmark separating posterior and anterior orbital swelling	Orbital septum
5	Astigmatism treatment	Cylindrical lens
6	Best differentiator of trachoma from other diseases	Herbert's pits
7	Best initial step in management of amblyopia	Orthoptic treatment / patching normal eye
8	Best minimally invasive and safest cataract surgery	Phacoemulsification
9	Cause of optic nerve compression	Thyroid disease
10	Chronic diabetes with blurring due to ischemia	Maculopathy
11	Convergent squint with 6/60 and 6/6 visual acuity treatment	Cycloplegic refraction and orthoptic treatment
12	Correct pathway of aqueous humor	Posterior chamber → anterior chamber → trabecular meshwork → canal of Schlemm
13	Diabetes causes poor vision through ischemia in	Maculopathy
14	Diabetic patient with poor vision and absent red light reflex on fundoscopy	Hyperdense/mature cataract
15	Difference between preseptal and orbital cellulitis	Ocular motility is not affected in preseptal cellulitis
16	Differentiate aneurysmal CN III palsy from diabetic oculomotor palsy	Pupil is dilated
17	Diplopia occurs in which condition	Adult with restrictive myopathy due to thyroid disease
18	Diplopia recall item	Eye invasion [unclear]
19	Emergency surgery in thyroid exophthalmos is indicated in	Optic nerve compression
20	Eye movement test is performed to detect	Angle of squint [unclear]
21	Feature not found in trachoma	Photophobia
22	Fundoscopy with cherry red spot diagnosis	Central retinal artery occlusion
23	Head turn is seen in which refractive error	Astigmatism
24	Immediate treatment of angle-closure glaucoma	IV mannitol
25	Lower lid mass for 3 months, not hot and no pus, treatment not beneficial	Oral antibiotic
26	Main sign differentiating orbital cellulitis from preseptal cellulitis	Proptosis
27	Most common organism causing congenital cataract	Rubella
28	Most common site of allergic polyp	Ethmoid / lateral nasal wall [source note corrected]
29	Non-tender non-hot lower lid mass best treatment	Hot compress with steroid
30	Not an indication for CT in diplopia	Diabetic patient with 3rd nerve palsy and preserved pupillary reflex
31	Old patient with sudden unilateral painless loss of vision and retinal hemorrhage on fundoscopy diagnosis	Central retinal vein occlusion
32	Painful localized red eye with negative Schirmer test treatment	Topical NSAID + topical steroid
33	Painful tender localized upper eyelid swelling diagnosis	Stye
34	Painless progressive loss of vision diagnosis	Cataract
35	Papilledema is a finding in	Cavernous sinus thrombosis
36	Polyopia image on retina is seen in	Keratoconus
37	Pre-auricular lymphadenopathy occurs in	Adenoviral conjunctivitis
38	Pseudosquint cause	Myopia
39	Red eye due to herpes simplex keratitis treatment	Acyclovir
40	Red eye due to seasonal allergy treatment	Olopatadine
41	Red eye with pain and misdirected eyelash diagnosis	Trichiasis
42	Small non-tender eyelid lesion with no progression diagnosis	Chalazion
43	Structure transmitting function of ciliary muscles to lens	Lens zonule
44	Sudden loss of vision with painful red eye, nausea and vomiting diagnosis	Primary acute angle-closure glaucoma
45	Sympathetic system effect in fright	Pupillary dilatation
46	Test for monocular diplopia	Slit lamp / visual acuity [unclear]
47	Thyroid eye disease true statement	Usually bilateral and symmetrical
48	Treatment not used for chemical conjunctivitis	Vitamin A

#	Fact cue / question	Answer
49	True regarding proptosis	Diagnosed by Hertel exophthalmometry >17 mm
50	True statement about primary open-angle glaucoma	Optic nerve atrophy
51	Typical case of acute unilateral vision loss	Acute angle-closure glaucoma
52	Ulcerated lesion in lower eyelid treatment	Excision

Orthopedics (73 questions)

#	Fact cue / question	Answer
53	3-part fracture shaft humerus with displacement best treatment	ORIF with plates and screws
54	3-year-old child with crying, painful extended/pronated elbow unable to flex	Pulled elbow
55	5-year-old painless limp, limited abduction, LCPD true statement	Surgery during fragmentation stage is most effective
56	Ankle trauma: skeletal survey indicated in	Unconscious patient / fall from height
57	Ankle trauma: surgery indicated in ankle fracture when	Tibiofibular diastasis
58	Best treatment for unstable intertrochanteric fracture	Proximal femoral nail
59	Cause of non-union	Not immobilized completely
60	Child with red swollen metaphysis and high WBC	Acute hematogenous osteomyelitis
61	Chondromyxoid fibroma true/false item	Pathologically made from mucinous material and cartilage
62	Chronic osteomyelitis with sinus in proximal thigh management	All of the above / antibiotics, debridement, sequestrectomy
63	Club foot true statement	Treatment is usually non-operative
64	Compartment syndrome best treatment	Fasciotomy
65	Complication not seen in closed fracture	Infection
66	Complication of lateral condylar fracture if left capsized	Non-union
67	Complication of scaphoid fracture	Avascular necrosis of proximal third
68	Cubitus valgus with pain/tingling over medial forearm treatment	Anterior transfer of ulnar nerve
69	Cubitus varus treatment	Wedge osteotomy with base located laterally
70	DDH at 2.5 years treatment	Open reduction with Salter osteotomy
71	Erb palsy muscle not affected	Interosseous muscles of the hand
72	Fall backward on outstretched hand with shoulder pain and prominent acromion diagnosis	Anterior shoulder dislocation
73	False regarding genu varum	Assessed by intermalleolar distance
74	False regarding menisci	It is hyaline cartilage
75	False regarding Perthes disease	Onset >10 years
76	Female athlete with stress fracture: important history	Menstruation/nutrition
77	Femoral neck fracture in elderly after 6 weeks best treatment	Hemiarthroplasty
78	Finding not seen in DDH	Avascular necrosis
79	Fracture followed by tense forearm and pain on stretching	Compartment syndrome
80	Important factor for fracture healing	Immobilization
81	Inability to lift shoulder with positive abduction paradox test diagnosis	Complete rotator cuff tear
82	Incorrect regarding nerve injuries	Crush injury results in axonotmesis
83	Initial step after fall from height with unconsciousness and ankle swelling	Radiological assessment of knee, hip, spine and base of skull
84	Interarticular displacement fracture treatment	ORIF
85	Legs straight with medial malleoli touching and knees >10 cm apart diagnosis	Genu varum
86	Lysalic fracture displaced >2 cm with skin bulge treatment	ORIF
87	Minimally displaced clavicle fracture best treatment	Arm sling and analgesia
88	Monteggia fracture-dislocation best treatment	Restore length of ulna / ORIF ulna
89	Most common complication of intertrochanteric fracture	Malunion
90	Most common dislocation of hip	Posterior dislocation
91	Most emergent treatment of talar fracture	Type III talar fracture
92	Most sensitive sign for DDH in 6-month-old child	Limited abduction in flexion
93	Nerve injured with medial epicondyle humerus fracture	Ulnar nerve
94	Nerve injury in anterior shoulder dislocation	Axillary nerve
95	Not a feature of rocker-bottom foot	Flat sole
96	Not true about calcaneal fracture	Results in avascular necrosis
97	Not true about DDH	Hip X-ray confirms the diagnosis [not true in early infancy]
98	Osteoid osteoma true statement	Responds to NSAIDs well
99	Overuse shoulder pain with difficulty elevation and responds to conservative treatment	Acute calcific cuff tendinitis

#	Fact cue / question	Answer
100	Painful twisting ankle during football: best investigation	MRI or plain X-ray film [source conflict]
101	Painful wrist with positive Finkelstein sign diagnosis	De Quervain disease/tenosynovitis
102	Pelvic ring injury with grade 3 anteroposterior compression likely needs	Blood transfusion
103	Position for immobilization of metacarpal bone fracture	Safe position
104	Proper immobilization includes	Joint above and below the fracture
105	Radiologic abnormality diagnostic of ankle syndesmosis disruption	Tibiofibular clear space >6 mm on AP
106	Reconstruction in lateral collateral ligament tear means	Putting the tendon in place of the ligament
107	Rotator cuff tear clinical diagnosis	Abduction paradox and drop sign
108	Severely displaced Colles fracture treatment	Closed reduction and splint below elbow
109	Shoulder dislocation suspected after epilepsy	Posterior shoulder dislocation
110	Simple bone cyst aspiration finding	Straw-coloured fluid
111	Stable intertrochanteric fracture definition	Lack of lesser trochanter fracture
112	Talipes calcaneovalgus differs from talipes equinovarus except	Toes touching the ground
113	Tennis elbow treatment	Physiotherapy
114	True about chondroblastoma	Affects epiphysis, usually proximal humerus/femur/tibia
115	True about chondroma	Usually asymptomatic
116	True about giant cell bone tumor	Affects mature bones; rare before physeal closure
117	True about ivory exostosis	Affects all bones including skull
118	True about osteoblastoma	Possible malignant change risk
119	True about osteochondroma	Stops growing at end of growth of affected bone
120	True about osteoid osteoma	No risk of malignant transformation / responds to NSAIDs
121	True about rocker-bottom foot	Talus is vertical
122	Undisplaced 3rd metacarpal fracture treatment	Splint immobilization in safe position
123	Undisplaced distal radial fracture treatment	Backslab below elbow
124	Undisplaced radius/ulna fracture in child treatment	Splint above elbow
125	Young patient with painful shoulder and history of overuse	Acute tendinitis

Urology (77 questions)

#	Fact cue / question	Answer
126	12-year-old unable to void after football, suprapubic pain/mass investigation	Retrograde urethrogram
127	5 cm renal mass on ultrasound next step	CT
128	70-year-old with erectile dysfunction second step	Send HbA1c/exclude diabetes
129	Acute prostatitis presentation	Acute prostatitis
130	Acute urinary retention matching	Acute suprapubic pain with fever and inability to void
131	Admission indication in ureteric colic not true	No response to analgesic after half hour [source says not indication]
132	Azoospermia with lack of secondary sexual characteristics matching	Hypogonadism
133	Bladder tumor with no muscle in biopsy: next diagnostic step	Repeat cystoscopy with TURBT
134	BPH does not cause	Cancer
135	BPH on tamsulosin + finasteride with PSA 3.2 management	Follow up yearly
136	BPH with IPSS 19 management	Severe LUTS disease
137	Cause of LUTS matching	Distal ureteric stone or recurrent UTI
138	Causes acute urinary retention	Acute cystitis
139	Chest pain in ED after Viagra management	Do not give nitrate in next 72 hours
140	Chronic prostatitis may cause	Epididymitis
141	Chronic urinary retention matching	Flaccid neurogenic bladder
142	Diabetic with HbA1c 14% and urinary symptom diagnosis	Urodynamic study
143	Diagnosis of acute pyelonephritis	Urine culture and sensitivity
144	Differential diagnosis of left ureteric colic except	Acute appendicitis
145	Does not occur in acute urinary retention	Overflow incontinence
146	Drug class not used in BPH	Phosphodiesterase stimulator
147	Drug that selectively inhibits 5-alpha-reductase type 2	Finasteride
148	Elderly smoker with hematuria and LUTS diagnosis	Urinary bladder urothelial carcinoma
149	Emergency management of pyonephrosis	Open drainage of kidney
150	Epidemiology of stone disease true statement	Urinary stone recurrence rate approaches 50% at 10 years

#	Fact cue / question	Answer
151	Evaluation of urinary retention true except	CVA/neurogenic bladder [source correction]
152	Functional unit of lower urinary tract	Detrusor muscle
153	Genetic inheritance of BPH	Autosomal dominant
154	Gold standard imaging for renal mass	CT scan
155	Gold standard investigation for renal mass	CT with or without contrast
156	Grade I VUR management not correct	Antimicrobial drugs followed by chronic suppressive therapy for 6 months or more
157	Gross hematuria and suprapubic bruising after car injury diagnosis	Extraperitoneal bladder laceration
158	Hematuria, anemia, pyrexia and hypertension diagnosis	Renal cell carcinoma
159	Indication for renal biopsy	Renal lymphoma
160	Male infertility with azoospermia and normal FSH cause	Normal FSH level without obstruction [unclear]
161	Male with suprapubic pain, fever, rigors, pus urine: investigation	Voiding cystourethrogram or CT scan [source conflict]
162	Most common finding on physical exam in male infertility	Left varicocele
163	Most common type of renal cell carcinoma	Clear cell carcinoma
164	Non-muscle-invasive bladder cancer treatment	TUR + BCG
165	Not a complication of BPH	Renal stone
166	Not a complication of hormonal therapy for prostate cancer	Hypoglycemia
167	Not a risk factor for bladder cancer	Hemodialysis
168	Not a risk factor for RCC	Simple cyst
169	Not established risk factor for RCC	Diabetes
170	Not indicated for admission	No response to analgesia after 1/2 hour
171	Not surgical indication for BPH	Urinary retention
172	Perineal trauma with blood at meatus and scrotal ecchymosis diagnosis	Bulbar urethral injury
173	Perirenal abscess not true	Mainly caused hematogenously
174	Postrenal anuria matching	Recent hysterectomy
175	Primary infertility of 2 years true statement	Both male and female partners should be assessed
176	Primary infertility with normal sperm parameters matching	Female factor infertility
177	Primary infertility with recurrent testicular pain, oligospermia and normal FSH cause	Varicocele
178	Pyonephritis/pionephrosis treatment	Percutaneous drainage
179	Pyospermia true statement	All of the above
180	Receptor in prostate and bladder neck	Alpha-1a receptor
181	Recurrent UTI in child true statement	Voiding cystourethrogram is gold standard for diagnosis
182	Renal stone not true statement	Most common in white female
183	Renal stone treatment not included	Anticoagulant
184	Renal trauma with 2 cm laceration and small perinephric hematoma no contrast leak management	Conservative management and follow-up
185	Scrotal mass hard, non-tender: treatment	Inguinal orchidectomy
186	Secondary infertility with azoospermia matching	Serum FSH, LH, and testosterone
187	Sudden erectile dysfunction after antidepressants second step	Reassurance that it is mostly psychogenic in nature
188	Sudden painful red swollen testis diagnosis	Torsion
189	Trauma to loin with hematuria and loin pain/ecchymosis initial investigation	CT scan with contrast
190	True about renal cell carcinoma	Can occur in von Hippel-Lindau disease
191	True about watchful waiting	None of the above
192	True regarding bilharziasis	Due to eggs in subepithelial layer of bladder
193	UPJ case not correct	Often bilateral
194	Uroselective alpha blocker	Alfuzosin
195	Vesicoureteric reflux stage not true	Stage 3 has clubbing of calyces
196	VUR epidemiology not true	Usually diagnosed after a child has UTI (70%)
197	VUR management not true	Stage 4 with preserved renal function treated conservatively
198	VUR not true statement	3/4 are boys
199	VUR true statement	Offspring of affected parent has up to 50% risk
200	VUR with mild-moderate dilation of ureter and pelvicalyceal system grade	Grade III
201	Wrong about urethral injury	Can be catheterized
202	Young patient with non-tender scrotal mass diagnosis	Seminoma

Medicine (19 questions)

#	Fact cue / question	Answer
203	A 45 year old with 20 year history of IDDM (insulin dependant DM) presents with progressive leg swelling, abdominal distension, irritability, itching, etc.	D. CKD
204	A 62 year old man presents with sudden anuria on a background history of several weeks of 'not passing much urine'. He denies dysuria or haematuria but admits to having a poor stream for many years. He is normotensive and otherwise looks well and has no systemic symptoms. What is the best initial diagnostic investigation'	D. Renal ultrasound scan
205	A child presents with features of nephrotic syndrome. +++ on urine dipstick. Electron microscopy shows podocyte fusion.	A. Minimal change nephropathy
206	A patient has passed many black tarry stools. His Hg is low, everything else was normal.	IDA
207	A patient is followed closely by her nephrologist for stage IV chronic kidney disease associated with focal segmental glomerulosclerosis. Which of the following is an indication for initiation of maintenance hemodialysis' A.Acidosis controlled with daily bicarbonate administration B.Bleeding diathesis C.BUN greater than 110 mg/dL without symptoms D.Creatinine greater than 5 mg/dL without symptoms E.Hyperkalemia controlled with sodium polystyrene	B.Bleeding diathesis
208	A patient was previously ok, had CT with contrast done, and then he developed azotemia (elevated BUN and creatinine) and some signs and symptoms	C. Acute tubular necrosis
209	A patient with anemia; blood film showed spherocytes and family history of the same condition.	Hereditary spherocytosis
210	A pregnant lady complains of fever and dysuria. She’s previously had a self limited reaction to penicillin but otherwise her past medical history is insignificant. Given that the organism responsible is sensitive to all of the following, which antibiotic do you choose'	E. Gentamycin
211	All of the following is a function of the tubulointerstitial compartment of the kidneys except	D. ultrafiltration of the blood coming to the kidneys
212	An old man presents with confusion, irritability, but no fever. He has a tender lower abdominal mass. His creatinine is 4.3 mg/dL. What do you do next'	A. Ultrasound of the abdomen
213	CML is characterized by all of the following except	E. Chronic phase of CML doesn’t require treatment
214	Patient previously had pancytopenia (all 3 cell lines low). Now has blast cells in peripheral blood.	MDS
215	Which of the following antibiotics is contraindicated in pregnancy'	A. Ciprofloxacin
216	Which of the following cysts is premalignant'	C. Cyst in patient on dialysis
217	Which of the following does not cause tubulointerstitial nephritis'	C. Post-streptococcal glomerulonephritis
218	Which of the following forms of diabetic ESRD is reversible'	A. Hyperfiltration
219	Which of the following is the most common cause of death in a patient on dialysis'	B. Myocardial infarction [source]
220	Which of the following is the most common cause of end stage renal disease'	A. Diabetic nephropathy
221	Which of the following is true about complicated UTI'	B. Most common source of gram negative sepsis

Pediatrics (119 questions)

#	Fact cue / question	Answer
222	(It was an extremely clear case of Diabetic Ketoacidosis) There was Ketone (+++). What is the most likely cause' a.Diabetic Ketoacidosis b.early onset type 2 diabetes	a.Diabetic Ketoacidosis
223	.Which of the following regard as a part of new Ballard score to assess gestational age '(dubara)A.subcutaneous fat	E. square window test
224	2 day year old neonate presented with respiratory distress, and imaging showed an inflated area of lung tissue with mediastinal shi (I dont remember the exact features but the diagnosis was congenital lobar emphysema). Whats the treatment'(lobectomy)	d. Lobectomy
225	4-day old presents with dark-coloured stool. Which of the following most likely indicates severe Bowel Ischaemia'(dubara)	b. Septic Shock
226	5 days history of fever with maculopapular on trunk and extremities, occasionally disappears.	Erythema infectiosum
227	A 10 year old case of pancreatic insufficiency and short stature with clubbing. Height and weight is falling behind but stunting was noticed at the age of 6. Cystic Fibrosis	weight is falling behind but stunting was noticed at the age of 6. Cystic Fibrosis
228	A 10 year old patient with a recent history of fever and URTI a while ago and petechiae and purpura all over the body. Their examination and history is otherwise unremarkable and laboratory investigations show CBC normal except for ITP ~10,000. What is the management'	A. No intervention as it’s a self limiting disease
229	A 13-year-old female was brought to the pediatrics consultation ward by her parents who were worried that she was “falling behind” in her growth. She had not yet developed secondary sexual characteristics, she had broad chest, low posterior hair-line. What is the next step in management'(dubara)	c. Chromosomal Abnormalities
230	A 2.4 kg neonate presents with a herniation of bowel through a defect to the right of the umbilicus, associated with necrosis (dont remember') and multiple atresias. Which of the following is important regarding survival in these patients' a.Delayed return of bowel function b.state of bowel [source] c.Associated anomalies D.weight of the child e……	b.state of bowel [source]
231	A 3-month-old girl is brought to the Accident and Emergency department by her parents aer having a fit at home. She is unconscious on arrival and then has a prolonged apnoea that requires intubation. Closer examination reveals that she has several small round bruises on her back and on her chest which are each about 1 cm in diameter. G	back and on her chest which are each about 1 cm in diameter. G
232	A 3-week old infant has a barium upper gastrointestinal series to evaluate vomiting. The Duodenojejunal flexure is found to be to the right of the midline as well as more caudal and anterior than anormal ligament of Treitz. The child is seen to reflux barium spontaneously to the level of the mid-thoracic esophagus. You would recommend which of the following'	b. Emergency laparotomy.

#	Fact cue / question	Answer
233	A 3-year-old child is brought to your clinic with a fever and stiff neck. You are quite certain the child has meningitis. When you discuss the need for a spinal tap and antibiotic treatment, the parents refuse permission, saying, "We'd prefer to take him home and have our minister pray over him."	D. the physician is justified in seeking legal authority (in the form of a court order of; authorization from a state child protection agency) to proceed with the procedure and; treatment of the child
234	A 32 weeks GA fetus presents with respiratory distress and massive ascites, his mother was a narcotic user, has premature rupture of membranes a week ago, and also had an abruptio placenta (the case didnt directly s ay this but it suggested it). Interrupted fetal lung maturation can occur with:(dubara) A.hydrops fetalis B.due to narcotic C.Premature separation of placenta D.premature rupture of membrane E.	A.hydrops fetalis
235	A 37-week neonate who was 4.5 kg and was born through C/S. Who had an open abdomen covered with a sac. His blood sugar level was 1.1mMol/L and had a temperature of 35.8 degrees. What is the cause of this hypoglycaemia'(dubara)	b. Due to high insulin secretion
236	A 4 week old child presented with vomiting that has been gradually worsening and had an upper GI series done. Which of the following is the most likely diagnosis'(Pediatric Surgery)	E. Pyloric stenosis
237	A 4-month-old girl presents with a swelling on her le leg. She is otherwise well and alert. An X-ray reveals a spiral fracture of her le femur. Her parents tell you that they don’t know how it occurred and that maybe her 18-month-old brother hit her. She has been seen by the orthopaedic team and is in traction, and they have already referred her to the Children’s Social Care Team and to ophthalmology. H REGARDING Acyanotic heart diseases	Care Team and to ophthalmology. H
238	A 5-year-old girl presents with a spiral humeral fracture. Her mother says that she tripped over the back step, but the girl looks nervous and says ‘No I didn’t, it was on purpose.’ D	over the back step, but the girl looks nervous and says ‘No I didn’t, it was on purpose.’ D
239	A 9 months old child that aer weaning developed diarrhea and was suspected of celiac disease. The parents suspected the gluten to be the cause and removed it from the diet. Now they want to confirm the diagnosis; how can they'	D.gluten challenge test
240	a case of 10 years old female presented for another reason, and on examination was noted to have a loud systolic ejection murmur at the le sternal border. The mother said the child is being followed up regularly by a cardiologist and they said it will resolve by itself no need management -small VSD.	-small VSD.
241	A case of a young child with fever of 5 days and petechiae and pupura. The parents are consanguineous. The laboratory findings were Hb 9 mg/dL, WBC 3000, and PLT 40,000. Which of the followings investigations do you send for' a.Bone marrow aspiration b.Bone marrow biopsy	a.Bone marrow aspiration
242	a case of dehydrated child aer diarrhea and vomiting, with skin turgor decreased , sunken fontanelle. Which of the following fluids is appropriate intially' A.dextrose (¾) with NS B.dextrose(½) with ½ NS C.NS	C.NS
243	A case of diarrhea with some dehydration	D.ORS
244	A case of hemophilia A on factor VIII replacement has poor control and continuous bleeding,(dubara) what’s the next step of management'	b. Factor VII replacement
245	A case presented with fever, and cutaneous bleeding. CBC was normal except for low platelets. The diagnostic work up should include evaluation for	Immune thrombocytopenia (marked ?not sure? in source)
246	A case was given with a motor delay. You suspect cerebral palsy. What is the next step in order to confirm the diagnosis' Clinical assessment Brain MRI … … …	Clinical assessment
247	A case with short stature, both mother and sister also had short stature, she had a large dysmorphic head, she had issues with gross motor skills, thought by the parents to be due to the large dysmorphic head, but the other areas of development were normal and the child was intelligent and well adjusted. She had rhizomelic shortening of the limbs and normal trunk height. Achondroplasia	height. Achondroplasia
248	A child born at term, vaginal delivery and aer 1 hour he has no health problems.	a. Vitamin K 1mg IM
249	A child developed cough, fever, conjunctivitis and a rash that spread from the head to the trunks. His pediatrician diagnosed him with Measles.	C. Vitamin A 100 000 to 200 000 IU.
250	A child had been brought to hospital by his parent and complaining of that their child was playing with a bottle of kerosene 3 hrs ago, aer 1 hr he started to have a cough, gagging and labouring breathing. During your examination the child was totally fine, what is your next step in managing'(dubara)	a. Admission to hospital and observation for 24 hrs.
251	A child presents with fever and respiratory features not responsive to medications and antibiotics. No hepatosplenomegaly or lymphadenopathy were noted. CXR shows a wide mediastinum. What’s the management'	E. Trucut direct biopsy
252	A child with bilateral cataract, hearing deficits, IUGR, and a murmur (systolic at le sternal border i think), thrombocytopenia, and some other features	B. Congenital rubella
253	A child with bowing of legs and seizure. On lab test he had a calcium level of 5.5mg (NR was given, was low).	a. Oral Vitamin D and intravenous Calcium
254	A child with generalised body stiffness, nocturnal myoclonus with a hyoersyrhmia EEG. He had no cherry red appearance. Which of the following is inappropriate'(dubara)	a. Benign and does not need treatment
255	A child with recurrent chest infections, other features suggesting large VSD. Which of the following is not correct regarding its management'	D. Reassure and follow up
256	A child with red eyes that has had fever and rash that started at the back of the head and spread to the trunk.(dubara)	c. Measles
257	A chromosomal analysis shows a child with 46,XY,t(14q21q). What’s the management'	D. Offer a chromosomal analysis for both parents to see if they carry a balanced robertsonian; translocation.; E. …
258	A mother is breastfeeding her child, got diagnosed with tuberculosis, which of the following regarding the breastfeeding is correct:(dubara)	c. Stop breastfeeding and receiving anti-Tb, continue breastfeeding aer 2 weeks
259	A mother wants to stop breastfeeding her 5 months old child because she’ll go back to work. You advice her all of the following in support of breast feeding except	E. You should not stop weaning with breast feeding at 6 months
260	A neonate presented with scaphoid abdomen, bowel sounds in the chest, and some other features that suggested congenital diaphragmatic hernia. The timing of surgery depends on:(Pediatric Surgery) a.o2 and acidosis b.Size of defect C.How many loops of bowel in intestine	a.o2 and acidosis

#	Fact cue / question	Answer
261	A neonate presents with features suggest potter syndrome, and has a mass covered by a sac (omphalocele). Which of the following is correct regarding management	B. Painting the sac and repairing it later at 1 year
262	A neonate presents with whitish vaginal discharge. What’s the cause'	C. Maternal estrogens
263	A newborn has bilateral cataracts and microphthalmia, intrauterine growth retardation, hemorrhagic skin lesions scattered throughout the body, and a harsh systolic murmur heard at the le sternal border and radiating to the lung fields.	a. Congenital Rubella
264	A newborn has hydrocephalus, chorioretinitis, intracranial calcifications, and anemia.	a. Congenital Toxoplasmosis
265	A newborn has microcephaly, intracranial calcifications, hepatosplenomegaly, and marked hyperbilirubinemia and thrombocytopenia.	a. Congenital Cytomegalovirus
266	A nurse does a developmental milestone assessment for a 5 months old baby. She determines he is developmentally appropriate hearing and speech. What were her findings'	B. Monosyllablic babbling
267	a patient drug user, history of syringe sharing in last 3 weeks with someone infected. Which of the following would most likely indicate hepatitis B in this patient, particularly in the first day of exposure'with early onset hepatitis B will be '(dubara)(handwriting file) A.HBS +ve , HBc IgM -ve , DNA +ve B.HBS +ve , HBc IgM +ve , DNA +ve C.HBS +ve , HBc IgM -ve , DNA -ve D…. E……	B.HBS +ve , HBc IgM +ve , DNA +ve
268	A patient with diagnosed Sickle cell disease (HbSS) presented with hemiparesis, and some other features. Which of the following is the appropriate acute managment'(dubara)	b. Exchange Transfusion
269	A patient with maculopapular blanching rash over the face, trunk and extremities, with sore throat and difficulty swallowing, tender cervical lymphadenopathy and tonsillar swelling with exudates.	C. Scarlet fever
270	a typical case erythema and r edness over breast ! ' A.mastitis. B.engorgement	Mastitis
271	a typical case of CMV, the diagnosis was given , most accurate diagnosis of CMV is by'(dubara) A.viral PCR B.viral culture C…. D…. E….	A.viral PCR
272	A typical case of infantile spasm , startle reflex , hypsarrythmia …. Treatment ' A.adrenocorticotropic hormone(ACTH) B.vigabatrin C.diazepam	A.adrenocorticotropic hormone(ACTH)
273	About child development, which one of the following start first' A.circle B.cross C.square D.triangle E.hexagon	A.circle
274	All features of TOF and boot-shaped mediastinum on X-ray.	TOF
275	All the following are indicators for a pathologic murmur except: A.grade II B.grade III C.repeated infection D…….. E…….	A.grade II
276	All the following are true regarding physiologic anemia of infancy EXCEPT(dubara)	E. iron deficiency is a contributing factor
277	An 18 hour old neonate has jaundice and is otherwise well. What’s the cause'	Incompatibility between blood group and child
278	An adolescent female patient presents with pallor, fatigue, and other anemia features, and the doctor suspects IDA. What should his next question be'	A. Ask about the amount of bleeding monthly
279	At what age should parents consult a pediatrician if their child is still not walking'	A. 18 months
280	case of cyclophosphamide lead to complication	Hemorrhagic cystitis; bone marrow suppression (both marked)
281	Case of down syndrome(all features of Down syndrome like sandal gap). DS has association with all except: A.Leukemia B.Atlantoaxial subluxation C.Crohn's disease D.Hirschsprung E.Alzheimer	C.Crohn's disease
282	Case of jaundice in a 3 year old child that’s otherwise well, Coomb’s negative, Hb 14, TSB 8.5 . Physiologic jaundice	. Physiologic jaundice
283	case of malrotation of duodenojejunum flexure with .. -upper endoscopy GI	Upper GI series / upper GI study (written in source as upper endoscopy GI)
284	Case with fever and malaise and tender cervical lymphadenopathy.	C. Mononucleosis
285	Case with uncomfortable sensation in the legs, has to occasionally move her foot to relieve it. -restless leg syndrome	-restless leg syndrome
286	Cases of cyanosis. All of the following indicates a cardiac cause except	c. Respiratory acidosis
287	child crying at night without any physical signs or symptoms , growing well…… -Infantile colic	-Infantile colic
288	Child stiffens when being picked up, is very jerky and also stiffens at sound. -Hyperekplexia	-Hyperekplexia
289	Child with flecks of blood , history of atopic asthma -cow milk diarrhea	-cow milk diarrhea
290	child with hypertrophic cardiomyopathy; what’s the mode of inheritance of disease'	D.autosomal dominant
291	Cow milk intolerance case.	Trial of hydrolyzed formula
292	Crossing legs, distant vacant flushed face, irregular breathing and sweating. -self-Gratification REGARDING Asthma	-self-Gratification
293	excessive crying child , with bulging fontanelle …… -Meningitis For each of the following cases of short stature, select the most appropriate diagnosis: A.rickets	-Meningitis
294	In children the diagnosis of pneumonia is made by	B-X ray
295	In neonate hypocalcemia least likely associated with'(dubara) A.seizures B.jitteriness C.high pitched cry D.laryngospasm E.carpopedal spasm	Carpopedal spasm
296	In the IMCI, which of the following features is used to assess dehydration and affects decision making' A.skin pinch	A.skin pinch
297	Jaundice in a child that’s 4 weeks old. Bunch of laboratory numbers. The gallbladder was not visualized on US. Biliary atresia	Biliary atresia
298	Jaundice since day 3 in a 4 week old child that hasn't responded to multiple exchange transfusions and phototherapy sessions. Conjugated bilirubin was low. crigler najar syndrome	crigler najar syndrome
299	Lactose intolerance case.	Stool pH
300	Long case, diagnosis of ASD strongly suggested by presence of wide splitting S2 with no variation with respiration. Which of the following is not true'	A. Most common congenital cardiac defect in pediatrics

#	Fact cue / question	Answer
301	Most common cause of otitis media in children: A.strep. pneumonia B.H. influenza C.moraxella catarrhalis D.mycoplasma pneumonia E……..	A.strep. pneumonia
302	Most common site of FB lodgement is' a.Right main bronchus b.Le main bronchus c.Larynx d.Trachea	a.Right main bronchus
303	Most effective treatment of poliomyelitis is:(dubara)	e. Bed rest and pain killer.; b. Fibrinogen (causes a prolongation of everything)
304	Most important initial investigation aer active resuscitation of neonate'	B. Blood glucose
305	newborn delivered at 26 weeks GA, and he hasn't been able to be taken off the ventilator. There is loud continuous murmur below the le clavicle: -large PDA. REGARDING …….. A……	-large PDA.
306	On Lab test, while PTT, PT and TT are all prolonged. What is the most likely diagnosis'	Fibrinogen
307	Patient presented with difficulty feeding, frothing and drooling, and imaging showed bilateral pulmonary aspirates, a gasless abdomen, and an NG tube that coils in the upper mediastinum. (Pediatric Surgery)	D. Esophageal atresia with tracheoesophageal fistula
308	Patient presents with short stature, genu valgum, abnormal gait, and radiography features of Coarse trabeculation of the diaphysis, other features and widening of the metaphysis of fibula, femur and tibia. Rickets REGARDING jaundice A.biliary atresia	femur and tibia. Rickets
309	platelets were 39000, other tests were normal A.post viral infection B.infectious mononucleosis C…… D…….. e.ITP a.pt increasing maybe only presentation b.Those with intracerebral hemorrhage have 30-50% mortality	a.pt increasing maybe only presentation
310	Seizure case. EEG showed hypsarrhythmia. What is the diagnosis'	Infantile spasm
311	Term male neonate presenting with tachypnea, intercostal retractions, cyanosis, grunting, tachycardia, and CXR showing bilateral faint haziness.	TTN (transient tachypnea of newborn)
312	The anomalies most commonly associated with omphalocele are	A. Cardiac anomalies.
313	the are the presenting symptoms ' -wheeze, cough, shortness of breath From the list of options below, please select the most appropriate initial action for each of the following scenarios concerning child protection issues. Each option may be used more than once.	-wheeze, cough, shortness of breath
314	The best management for moderate to severe croup is: (dubara)(handwriting file) A.oxygen B.nebulised epinephrine [source] C.inhaled steroids D.antibiotics E.	B.nebulised epinephrine [source]
315	The first line of management in mild to moderate bronchiolitis is: A.antibiotic B.hydration c.O2 D… E……	c.O2
316	the most common cause of obstructive sleep apnea syndrome (OSA) in young children is(dubara)	E. adenotonsillar hypertrophy
317	Triad of intussusception: A.colicy abd. pain, abdominal mass, constipation B.colicy abd. pain, abdominal mass, bleedig per recturm C.colicy abd. pain, abdominal mass, bilious vomiting D…… E……..	B.colicy abd. pain, abdominal mass, bleedig per recturm
318	typical case of Guillain-barre syndrome can be diagnosed by:(dubara)	b. Nerve conduction study and EMG
319	Ulcerative colitis disease case.	Colonic biopsy
320	what are the lines of treatment -bronchodilator and steroid inhaler.	-bronchodilator and steroid inhaler.
321	what are the triggers -exercise, old water, allergen	-exercise, old water, allergen
322	What is the most common presentation of Meckel’s Diverticulum in a 6 month infant '(dubara)	a. Intestinal obstruction
323	Which of the following could be harmful for a child with infantile colic' (dubara)(2020file) A.pacifier B.extensive investigation	B.extensive investigation
324	Which of the following describes the fine motor abilities of a two year old'	C. Eat with a fork [Source]
325	Which of the following diseases is associated with plethoric lung fields'	A. large VSD
326	Which of the following is a risk factor for pneumonia'	B. Asthma
327	Which of the following is best for galactosemia	C. Lactose-free fromula
328	Which of the following is correct regarding status epilepticus'	D. Management includes up to 2 doses of IV diazepam/lorazepam, then phenytoin maintenance,; and then consider anesthesia if still not controlled
329	Which of the following is correct regarding vitamin K deficiency'	C. Intraventricular hemorrhage is associated with high morbidity and mortality, with mortality rate in developing countries reaching 30-50%
330	Which of the following is indicated in uncomplicated meconium ileus'	A. IV fluid and gastrografin enema
331	Which of the following is not correct regarding diagnosis of malrotation'	The duodenojejunal junction being left of the spine excludes it
332	Which of the following is not part of the management of Thalassemia'	D. Elemental iron
333	Which of the following is the most common form of child maltreatment'(dubara)	d. Neglect
334	Which of the following medications is not used in the treatment of asthma'	A. Amoxicillin
335	Which of the following organs is spared in asymmetric IUGR'(dubara)	e. Brain
336	Which of the followings is the CSF finding of viral meningitis'(dubara) 15 4 Protein	b. WBC 100 neutrophils 30%, protein 40, glucose 60. 50
337	Which one is wrong ' A.rett is x linked dominant B.turner is monosomy C.marfan is autosomal dominant D.down syndrome triploidy	D.down syndrome triploidy; D. Incompatibility between blood group and child
338	Which one of following happen indicate secretory rather than osmotic: A.Reducing substance positive B.Higher stool volume C.ph 4.5 D.napkin rash E.improve with fasting	B.Higher stool volume
339	You refeed a severely malnourished child. Which electrolyte abnormality do you expect' A.Hyperkalemia B.Hypokalemia C.Hyperphosphatemia D.hypercalcemia	B.Hypokalemia

#	Fact cue / question	Answer
340	Young child (the age is important but I dont remember the exact age) presented with a febrile clonic seizure on one side of the body that then progresses to the other side, lasting more than 20 minutes, and occuring twice in 6 hours (i dont remember the exact frequency but the important thing is that it recurred). The patient has a family history of febrile seizures. His temperature was 105°F before the seizures. Which of the following factors increases their risk of developing epilepsy' A.complex febrile seizure B.fever preceding seizure C.Their age D.their family history e…….	A.complex febrile seizure

ENT (82 questions)

#	Fact cue / question	Answer
341	Adenoid pathology is due to	Size and infection
342	Adenoidectomy is important to prevent	Maxillofacial abnormality
343	Atrophic rhinitis true except	More common in males
344	Atticoantral chronic suppurative otitis media statement	Scanty offensive discharge
345	Basal fracture statement	Can be reduced under local anesthesia in adult
346	Bilateral blockage, anosmia, nasal discharge, postnasal drip, headache diagnosis	Bilateral nasal polyps
347	Bilateral vocal cord paralysis is common in	Thyroid surgery
348	Child unilateral nasal blockage/offensive discharge	Foreign body
349	Child with fever and severe earache	Acute otitis media
350	Child with stridor, toxicity and drooling diagnosis	Acute epiglottitis
351	Cholesteatoma matching	Bone erosion
352	Concerning allergic rhinitis true statement	Diagnosis usually based on history and clinical findings
353	Dry perforated eardrum matching	Myringoplasty is treatment of choice
354	Ear drop antibiotic matching	Otitis externa
355	Early glottic carcinoma best treated by	Radiotherapy
356	Epistaxis indicated for surgical intervention	Anterior and posterior nasal packing
357	Etiology of hoarseness true statement	Reinke's edema is associated with smoking
358	Eustachian tube opens in	Lateral wall of nasopharynx
359	False about otitis media with effusion	Tympanic tube is patent
360	False regarding imaging for neck mass	MRI for skeletal larynx
361	False regarding larynx anatomy	Framework of larynx is made from bone
362	False regarding larynx physiology	Straining is easy in people with vocal cord paralysis
363	False regarding multiple papilloma of larynx	Only involves glottis region / high risk malignancy [source conflict]
364	Farmer/smoker with hoarseness and thick vocal cord on endoscopy	Chronic laryngitis
365	Foreign body in nose true statement	Uncooperative children may need general anesthesia
366	Highest serum bacteria-specific IgE likely disease	Nasal polyposis
367	Indication for tracheostomy	Acute upper airway obstruction
368	Infant with stridor during feeding and normal cry diagnosis	Laryngomalacia
369	Most common cause of acute diffuse otitis externa	Pseudomonas aeruginosa
370	Most common cause of hearing loss in children in winter	Middle ear effusion
371	Most common complication of FESS	Synechiae
372	Most common site of osteoma	Frontal sinus
373	Most common/earliest presentation of septal deviation	Nasal obstruction
374	Muscle tension dysphonia characteristic	Vocal folds look edematous
375	Myringoplasty is plastic repair of	Tympanic membrane
376	Nerve that does not supply external ear	Chorda tympani
377	Newborn respiratory distress/feeding difficulty cause	Bilateral posterior choanal atresia
378	Not a complication of tracheostomy	Hypothyroidism
379	Not a protective mechanism in ear	Oval window
380	Not a symptom of ear disease	Presbyopia
381	Not a systemic cause of epistaxis	Severe septal deviation
382	Old diabetic with severe earache	Malignant otitis externa
383	Orbital swelling after URTI route of transmission	Thin ethmoidal wall
384	Postoperative tracheostomy care	Frequent suctioning and cleaning of tube
385	Purulent discharge, facial pain/headache, obstruction and fever after URTI	Acute rhinosinusitis
386	Recurrent epistaxis matching	Anticoagulant use
387	Scarlet fever	Rare complication of streptococcal infection
388	Self-limited epistaxis matching	Epistaxis in children

#	Fact cue / question	Answer
389	Sneezing is most common feature in	Allergic rhinitis
390	Soft wood occupational exposure increases sinonasal risk	Squamous cell carcinoma
391	Sore throat + fever, antibiotics did not relieve symptoms	Infectious mononucleosis
392	Sore throat with malaise and neck swelling	Diphtheria
393	Sore throat, stridor, fever, drooling, toxic diagnosis	Acute epiglottitis
394	Statement wrong about ear discharge	Ear syringing is main treatment in discharging ear
395	Structure with major role in sound amplification in ear	Ear drum
396	Sudden severe sore throat, fever, rigors, malaise, otalgia, cannot open mouth	Peritonsillar abscess
397	Suspected acute streptococcal tonsillitis	Sudden severe persistent sore throat and pyrexia with swallowing difficulty
398	Swimmer with severe earache	Acute diffuse otitis externa
399	Tonsillectomy indication	5 real attacks in each year of the last 2-3 years
400	Tracheostomy can cause pneumothorax when	Apical pleural injury
401	True about acute tonsillitis caused by streptococcus	Nowadays amoxicillin and amoxiclav are used
402	True about choanal atresia	More common in females
403	True about epistaxis	Mostly idiopathic
404	True about nose	Septum supplied by branches from external and internal carotid arteries
405	True about palatine tonsil in pharynx	Has 12-15 crypts
406	True about scarlet fever	Rare streptococcal tonsillitis with added features from a specific toxin
407	True regarding acute tonsillitis	Antibiotics are mainly used
408	True regarding diphtheria	Bull neck is characteristic feature
409	True regarding flu	Unlike common cold, onset of flu symptoms is sudden and fast
410	True regarding pure tone audiometry	Can be used to determine site of pathology
411	True regarding quinsy	All of the above
412	True regarding tubotympanic chronic suppurative otitis media	Scanty foul-smelling discharge
413	True statement about tympanic membrane	Tense small part and flaccid large part
414	Tubotympanic chronic otitis media true statement	Discharge is not foul smelling
415	Tumors of nose/paranasal sinuses true statement	Hard wood exposure increases adenocarcinoma risk
416	Unilateral nasal obstruction in child with foul purulent swelling	Foreign body
417	Unilateral polypoidal mass from lateral nose in 55-year-old man	Inverted papilloma
418	Unilateral polypoidal mass on lateral nasal wall diagnosis	Inverted papilloma
419	Unilateral vocal cord paralysis can occur due to	All of the above
420	Watery nasal discharge finding	CSF rhinorrhea or allergic rhinitis
421	When does pain disappear in otitis media	Perforation of the drum
422	White patch on vocal cord with hoarseness diagnosis	Leukoplakia